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Parent Perspectives on Barriers to Behavioral Health Services for Children With ADHD

Journal of Attention Disorders
2026, Vol. 30(9) 1091–1103
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DOI: 10.1177/10870547251412660
journals.sagepub.com/home/jad
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Abstract

Objective: The American Academy of Pediatrics (AAP) recommends behavioral therapy as first-line evidence-based treatment for young children with ADHD, and in combination with medications for children ages 6 years and older. However, a significant percentage of young children are receiving medication treatment for ADHD rather than behavioral treatment. There is limited research capturing parent experiences of barriers to implementing behavioral health services for young children with ADHD diagnoses. This paper seeks to fill this gap by exploring these perceived barriers among parents of children diagnosed with ADHD.

Method: Parents of children with ADHD diagnoses ($N = 29$) were invited to participate in four focus groups to discuss their experiences with accessing treatment for their child. Focus group participants were recruited from one Southeast city through an ADHD parental support group and nationally from parents attending a conference on ADHD.

Results: Three themes emerged as perceived barriers that impact ADHD treatment. Parents reported barriers that can be conceptualized as (1) misconceptions and stigma; (2) availability of financial and time-related resources; and (3) treatment messaging and provider coordination. These barriers may be more pronounced for parents of young children with ADHD living in the Southeast.

Conclusion: Parent responses about perceived barriers in this study suggest a need for better coordinated care of ADHD across places of service, providers, and families. These findings may inform future efforts to improve access to and utilization of evidence-based treatments for ADHD in the Southeast and nationwide. (*J. of Att. Dis. 2026; 30(9) 1091-1103*)

Keywords
ADHD, behavior therapy, parent, barriers

Introduction

ADHD is the most common neurobehavioral disorder of childhood (Danielson et al., 2018, 2024). Based on 2022 parent-reported data from the National Survey of Children's Health, 11.4% of children ages 3 to 17 years were ever diagnosed with ADHD (Danielson et al., 2024). The estimated prevalence of ADHD diagnoses increased from 4.4 million in 2003 (Visser et al., 2014) to 7.1 million in 2022 (Danielson et al., 2024). ADHD can emerge during early childhood, and approximately 2% of preschool-aged children (3–5 years) have an ADHD diagnosis (Danielson et al., 2024). ADHD, especially when untreated, can lead to impairment with social functioning starting in early childhood (Rabinovitz et al., 2016) and later health risk behaviors, such as substance use (Hogue et al., 2017). Many children with ADHD also experience academic challenges, including higher prevalence of suspensions or expulsions, including during preschool, (Zeng et al., 2021), which can lead to difficulties into adulthood (Aretouli, 2018).

The American Academy of Pediatrics (AAP) recommends behavioral therapy as first-line treatment for children ages 5 years and under with ADHD, followed by medications if warranted, and combined treatment of medication and behavioral therapy for children ages 6 years and older (Wolraich et al., 2019). However, more than half (54%) of children with a current ADHD diagnosis were taking medication in 2022, but less than half (44%) had received behavioral treatment in the last 12 months (Danielson et al., 2024). Medication treatment among children with ADHD showed an average annual increase of 7%

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from 2007 to 2011 to 2012 (Visser et al., 2014) and has remained stable or declined in recent years (Board et al., 2020; Danielson et al., 2023, 2024; Davis et al., 2020). Use of behavioral therapy has remained stable between 2016 and 2022 despite increasing prevalence of ADHD, which may indicate possible continued gaps in access to treatment (Danielson et al., 2024).

Most children diagnosed with ADHD receive some type of services and support to help with their behavior during their childhood (Danielson et al., 2018). Examples of support include individualized education programs (IEPs) and 504 school programs, but the best evidence is for behavior therapy, delivered as individual or group child therapy, or family therapy (Evans et al., 2018; Wolraich et al., 2019). Parent training in behavior management is evidence-based as the first treatment option with preschool age children with ADHD diagnoses (Bussing & Murray, 2014; Wolraich et al., 2019), has strong evidence up to age 12 years in combination with medication, and may also be beneficial as part of behavioral intervention for adolescents (Evans et al., 2018; Wolraich et al., 2019). Parent training is a key recommendation from practitioners, especially for parents with symptomology of ADHD (Halperin & Marks, 2019). Children who receive behavioral therapy, including parent training and behavioral classroom management, experience better academic, social, and familial outcomes (Daley et al., 2014; Pelham & Fabiano, 2008). However, a significant percentage of young children are receiving medication treatment for ADHD rather than behavioral treatment (Bannett et al., 2021; Visser et al., 2014, 2016), and only about one third of parents reported ever receiving parent training (Danielson et al., 2018), thus treatment is not aligned with the best evidence-based practice.

Barriers to behavior treatment are experienced by many families, can be unique to the child's environment and family context, and can range from individual level factors such as knowledge, beliefs, attitudes, perceived needs and stigma, to factors affecting the social network, or the larger community (Kappi & Martel, 2021). Parents report a lack of knowledge about available behavioral parent training or other parenting program advertising for their preschool aged children (Smith et al., 2015). Some studies have reported positive perceptions of the effectiveness of behavior therapy (or non-medicated approaches) from parents and adolescents (Krain et al., 2005; Lu et al., 2022).

Stigma associated with ADHD diagnosis and treatment also impacts parental decisions to initiate treatment (dosReis et al., 2010, Kappi & Martel, 2021). Qualitative studies highlight increased familial and social stress from receiving unsatisfactory support for their child's experiences with ADHD (Leitch et al., 2019) and medication use as treatment (Charach et al., 2014). Before initiating treatment, parents of preschool children report reservations to initiating medication for their children (Hart et al., 2018), and perception of behavior treatment can affect initiation and participation in parent training (Chako et al., 2017). Structural factors including service systems and insurance and administrative factors can also restrict access to treatment (Eiraldi et al., 2006). Financial barriers to treatment are a significant factor and were higher among families with children diagnosed at an early age (Nasol et al., 2019). Access to ADHD diagnoses and treatment in the U.S. varies by state and region, with states in the south having higher prevalence of ADHD diagnoses among children ages 3 to 17 years but less initiation of behavioral health treatment compared to those in the western states (Danielson et al., 2022; Visser et al., 2014).

Further, parents' engagement in treatment is affected by their prior experience with healthcare providers (Taylor et al., 2021). Lack of knowledge is a significant barrier documented in research (Kappi & Martel, 2021), and parents may depend on healthcare providers for receiving such knowledge. Some providers may not follow clinical guidance to provide referrals for parent training (AlRasheed et al., 2021; Bannett et al., 2021).

Extensive data exists to highlight the efficacy of available treatment options for children with ADHD diagnoses (Wolraich et al., 2019); however, gaps still exist in the literature about parent experiences with managing options and uptake of ADHD treatment, especially in early childhood (McKenna et al., 2023). This study investigates experiences with behavioral health services in a sample of parents of children diagnosed with ADHD, including a subset of children diagnosed between ages 2 and 7 years. The study aims to highlight parent-perceived barriers to seeking and receiving behavioral health services. Given that behavior treatment is evidence-based and recommended as the primary treatment for young children but continues to be used less often than medication treatment (Danielson et al., 2022), the study especially focused on behavioral therapy such as parent training, for their children with ADHD, including both logistical barriers such as access and social/emotional barriers such as attitudes and perception.

Methods

This study was reviewed and approved by the university's Institutional Review Board. All participants provided written informed consent prior to participating. Parents or caregivers (hereafter referred to as "parents") of children with ADHD diagnoses were invited to participate in one of four separate focus groups to discuss their experiences accessing treatment for their child, with a focus on behavioral treatment. Two focus groups were held in a major city in the Southeast with subjects recruited from a local ADHD parental support group with assistance from the group leader, and two focus groups were held with participants attending a national, ADHD-themed conference with

support from conference marketing and registration staff. Participants were recruited through flyers and e-mail blasts requesting attendance of parents of children diagnosed with ADHD who were either (1) between the age of 2 and 7 years old at the time of the study, to focus on children who were identified with ADHD in the early years, before adolescence, when behavioral treatment should be offered first before medication, or (2) had received their ADHD diagnosis in the last 5 years, regardless of age, to focus on families whose child received their diagnosis more recently and who could best remember their initial diagnosis and treatment experiences. All participants received a light meal or refreshments and were compensated $40 for their participation in the focus group.

Data Collection

In sum, 29 parents participated across the four separate focus groups. Thirteen parents participated in one of two focus groups held in the Southeast. One of the Southeast focus groups included seven parents with children who were aged 2 to 7 years and the other group included six parents of children diagnosed with ADHD in the last 5 years (regardless of age). Two focus groups were held nationally with 16 participants that lived predominately in the Western geographic region of the U.S. Of these 16 parents, 1 parent had a child aged 2 to 7 years, and 15 parents had a child who was diagnosed with ADHD in the last 5 years. Seventeen participants across all four focus groups broadly identified themselves as the children's parents, while 11 participants identified themselves specifically as mothers. Because the location and context of the focus groups (national conference versus local support group) may have influenced the type of participant, we heretofore will label the two locations as "National focus group" and "Southeast focus group."

All four focus groups were conducted in-person between May 2015 and November 2016 and lasted approximately 60 to 90 min. To provide background information about the sample, prior to the focus group discussion, parents were administered a brief survey that included child demographics, diagnosis, and medication treatment (see Table 1). The focus group guide covered topics to ascertain the varied and multifaceted experiences with accessing ADHD treatment options, with a focus on behavioral treatment. Question topic areas included: accessibility and availability of services; experiences with behavioral and medication treatment for their child; motivations for implementing behavioral therapy; and expectations about services received. Focus group facilitators did not have any conflict of interest or relationship to the focus group participants. All four focus groups were audio recorded and transcribed verbatim. Names of participants and their children were de-identified to protect the identity of participants.

Qualitative Coding

The codebook was designed using an iterative process following Template Analysis, a form of Thematic Analysis (Brooks et al., 2015), that balances a high degree of structure with the flexibility to adapt to the needs of the study and what arises in the data. The first step in designing the codebook was to identify and include important concepts and themes in the focus group guide; deductive codes were identified during a review of the available literature and guided by initial research questions. The next step included further refinement based on an initial read of all transcript data; this involved confirming relevant codes, identification of new inductive lower-level codes related to diagnosis experience, organization of codes and themes into meaningful clusters, and clarifying meaning by providing definitions for the codes. A set of researchers skilled in qualitative research (C.S. and L.F.) independently coded approximately 10% of the sample focus group data. Coding between the two researchers was compared to assess for a standardized application of the codebook to the dataset and increase reliability. Standardized application of codes meant both coders were assigning the same codes throughout the chosen 10% of the sample from the transcripts. When discrepancies in coding were identified, these discrepancies were discussed, and the codebook was further refined to ensure clarity. Following confirmation of the codebook design and the subsequent standardization exercise, focus group transcripts were coded using Atlas.ti by the independent coders (C.S. and L.F. for the Southeast focus groups and C.S. and A.H. for the National focus groups).

Data Analysis

Data analysis of the four focus groups involved a rigorous application of thematic analysis (completed by B.T. and A.D.). Thematic analysis is a widely utilized qualitative method for organizing or highlighting recurrent themes, patterns, and connections between interview or focus group data (Braun & Clarke, 2006; Kiger & Varpio, 2020; Nowell et al., 2017). The application of thematic analysis for this study involved an in-depth review of the coded transcripts (by B.T. and A.D.), identifying generalized, emergent themes, and organization of defined themes across the four focus groups (see Table 1). Emergent themes were identified and refined inductively and informed by available literature on recommended ADHD behavioral intervention therapies and barriers to evidence-based treatment (Wolraich et al., 2019). An inductive approach occurs when themes are derived from the actual data, as opposed to identified a priori (Kiger & Varpiro, 2020). This inductive process included regular meetings with the research team (B.T., A.D., A.S., and C.S.) to

Table 1. Organized Themes and Patterns from Parent Focus Groups.

Defined theme	Type of code	Associated top-level code(s)	Associated lower-level code(s)
Parent misconceptions and stigma	Deductive	Attitudes from others toward child/family	• Stigma: child's behavior • Stigma: social • Attitudes: general
	Inductive	Diagnosis experience	• Awareness of potential problem/initial concerns • Identification and referral (who first diagnosed/type of provider/time to diagnosis) • Initial reactions to diagnosis
	Deductive	General knowledge about ADHD treatment	• Parent knowledge about ADHD and treatment • General knowledge about ADHD and treatment
Access and availability of treatment	Deductive	Attitudes toward treatment	• Parent attitudes toward treatment
	Deductive	Category of treatment	• Behavioral therapy (e.g., parenting, individual, group, social skills, etc.)
	Deductive	Types of treatment	• Alternative therapies/methods (e.g., natural, homeopathic remedies such as diet/nutrition, physical activity; brain exercise programs)
	Deductive	Reasons to stop treatment Payment/Cost	• No lower-level codes
Treatment messaging and provider coordination		Experiences with treatment	• Positive experiences with treatment • Challenges/struggles with treatment • Treatment management (medication management, navigation/coordination, advocacy, etc.)
	Deductive	Experiences with place of service (POS)	• Type of POS: Healthcare system (anything related to their experiences with the healthcare system, including both medical and mental health providers) • Types of providers seen • Services available • Positive experiences with POS • Challenges/struggles with POS

organize and confirm emerging themes. Saturation in both data collection and analysis largely depends on how the organized themes are connected to (1) the codebook design, (2) the framework used to support the research question, and (3) contributions to the field (Constantinou et al., 2017; Morse, 2015).

The inductive, iterative approach applied to all four focus groups supported the identification of emergent themes to answer the research question. A point of saturation was reached when new themes were no longer emerging in the analysis. Participants' responses across the four focus groups indicated sufficient commonalities and uniqueness in treatment experiences. Themes common across groups and those most relevant for the Southeast versus the national groups were noted. Themes were consistent among the two Southeast focus groups (with one focus group including children 2–7 years of age and one focus group with children diagnosed in the last 5 years, regardless of age).

Results

Descriptive Characteristics

Twenty-nine parents reported on 32 children with ADHD, as some parents reported having more than one child with an ADHD diagnosis in their household. Ages of children and length of time since diagnosis varied across all focus groups. The median age of participants' children was 10 years, with 62% of children between the ages of 6 and 10 years. Fifty-three percent ($n = 17$) of children were given an ADHD diagnosis between the ages of 2 and 7 years. Seventy one percent of children were identified by the parent participant as male ($n = 23$). Over half ($n = 18$, 53%) of

all children's ADHD diagnoses were provided by child psychologists, followed by pediatricians ($n=8$, 24%). Approximately 56% ($n=18$) of children were taking medication at the time of the focus group, and 9.3% ($n=3$) of children had previously taken medication but not currently at the time of the focus group. Table 2 provides more detailed information on child demographics.

Perceived Barriers to Receiving Behavioral Health Services for ADHD

Following the thematic analysis of both deductive and inductive codes, multiple themes emerged as perceived barriers to seeking or receiving behavior treatment for children with ADHD diagnoses, regardless of child's age when diagnosed. Parents also discussed medication treatment, but for the purpose of this study, the analyses focused on behavior treatment.

Parents of children aged 2 to 7 years from the Southeast focus groups more often reported barriers to ADHD diagnosis and behavior treatment compared to parents in the National focus groups. These barriers can be conceptualized as (1) misconceptions and stigma regarding ADHD; (2) access and availability of behavioral treatment; and (3) coordination and messaging.

Misconceptions and Stigma Regarding ADHD

When parents reported their first observations of ADHD symptoms, hesitancy around "knowing" and misconceptions around age-appropriate behaviors sometimes preempted – or even delayed – a clinical diagnosis and treatment. These observations were based on witnessing their child's behavior or reports from others (such as their child's teacher). Apprehension and related misconceptions regarding ADHD were reported by parents in both National and Southeast focus groups. One parent in a National focus group described her and her husband's initial denial of their child's behavior being attributed to ADHD, saying,

> So my husband and I were sort of in denial but we knew . . . I mean, I knew in my heart that there was something going on. (National focus group)

A parent in a Southeast focus group attributed their child's hyperactivity to stereotypes based on masculine and feminine expectations, dismissing a possible diagnosis. One parent noted,

> Um, but, yeah—we, for a long time, we figured, ADHD. He's a boy. He's young, you know what I mean? We really didn't, I guess, acknowledge it. (Southeast focus group)

Table 2. Demographic Characteristics of Participants' Children with ADHD Diagnoses.

Descriptives	Count	Percentage
Age of child (years)		
0–5	1	3
6–10	20	62
11–15	7	22
16 and older	4	13
Sex of child		
Male	23	71
Female	9	29
Region		
Northeast	2	6
Southeast	15	47
Southwest	1	3
West	14	44
Who provided diagnosis		
Psychologist	18	53
Neurologist	3	9
Neuropsychologist	2	6
Psychiatrist	2	6
Pediatrician	8	24
Other	1	3
Diagnosed between ages 2 and 7 years		
Yes	17	53
No	15	47
Receiving medication therapy		
Yes, currently	19	59
Never	10	31
In the past	3	9

Note. Table includes total count of children ($N=32$) reported by parents who participated in the focus groups ($N=29$); some parents reported more than one child with an ADHD diagnosis. Four children received diagnoses from more than one provider. "Other" refers to "Doctor." All percentages presented in this table may not add up to 100 due to rounding.

Another parent in a National focus group recalled how his son showed characteristics of hyperactivity, and assumed his behavior was warranted due to age. He stated,

> Um, at daycare, when my 11-year-old who is diagnosed with ADHD was about 5 – actually, 4 – there were lots of concerns about his impulsivity, inability to stay focused, and we just sort of let it . . . thought it was age-appropriate behavior . . . (National focus group)

Parents in both of the Southeast focus groups reported a negative reaction to the ADHD diagnosis. These parents identified feeling uncomfortable or saddened by their child's diagnosis, especially with fears of societal stigmas. One parent described an emotional response to their child now being "labeled" from the diagnosis, stating,

> And I hear ADHD and all of a sudden- this is my first encounter with the diagnosis- and I feel how my eyes are forming tears and what I think was . . . this was my thinking . . . he's labeled. He'll be labeled for the rest of his life, and this was very hard for me to digest . . . I suppose many parents struggle with this. (Southeast focus group)

One parent discussed the presumed stigma of parental blame associated with her child's diagnosis and struggle to seek assistance. She recalled a time when she reached out to her local community service board[1] for advice:

> And, so when I called the Community Service Board, a lovely lady answered . . . from then on, I felt like I had a support, but it was really hard for me to say that I needed help, and to say such a thing that I thought my child was experiencing 'cause she's a kid. And then, sometimes, to be honest, people don't realize it, don't wanna admit it—sometimes it's reflecting on you, is what you're feeling like. (Southeast focus group)

Access and Availability of Behavioral Treatment

When discussing the ability to seek out and receive treatment for their child, parent responses indicated barriers related to access and availability of treatment that stemmed from the lack of financial resources and time needed for treatment. Insurance coverage and the cost of treatment were reported as financial barriers to accessing treatment; many parents' insurance plans had a finite number of allowed visits for behavior treatment. One parent recalled,

> And it almost makes it impossible, now, because, now my insurance is changing . . . depending on how they're gonna bill it, won't be able to afford to take her if she needs to go, because you know, you're dealing with co-insurance, you're dealing with co-pays, and then you're also dealing with medicine . . . And like, my insurance only allowed so many visits. (Southeast focus group)

Another parent spoke about how they felt their insurance was a barrier to treatment, as well as feeling a sense of isolation rather than support from their child's psychologist. She noted,

> But you're basically out there . . . our insurance paid a little but we paid a significant amount of money for it and then she's [child psychologist] like, 'I can't treat you. Sorry' . . . so it's like you really are on your own to navigate the system, to see what else is out there. (National focus group)

Parents also spoke about the increased cost and time challenge that comes with behavioral therapies that require family engagement. When asked about integrating behavioral therapy, a parent in a Southeast focus group said, "we haven't been able to afford that so that hasn't been an option." Family engagement and training is considered the foundational component to behavioral treatment, especially for young children. Parents expressed frustration with the limited amount of time available in the day to attend family therapy with their child or to integrate tools at home during limited evening hours after school and work. One parent in a National focus group likened the time challenge that comes with treatment management, particularly with navigating various treatment while the child is at school, as "a nine to five job of just advocating and busting down doors."

Another parent in the Southeast expressed the time challenge being a barrier to their child's treatment even when treatment was working. She noted,

> . . .Once a week, both parents had to be present for quite a while. So we had to [use] sick leave or what-whatever leave from work. It required 30 minutes driving to the place. And then, respectively, driving our son back to school and going in the opposite direction to go to work. And we diligently did it for a year, and . . . it help [sic] him . . . at one point we decided it was really too much because the stress level in the family was great . . . it was really impossible. There was a toll. (Southeast focus group)

This parent described satisfaction with behavioral treatment services yet could not continue due to consistent conflicts with work and time spent attending appointments. Navigating time off from work became an undue burden that the family felt they simply could not sustain. A parent in a National focus group summarized the access and availability-related burdens in a similar way:

> So the decisions that you're making have a large consequence on your home life and how you're going to pay for all of it because you have no support to do it. (National focus group)

Treatment Messaging and Provider Coordination

One National focus group participant identified ADHD treatment as a "decision-making matrix," highlighting how parents may perceive multiple directions and entryways for treatment that often intersect with one another, and reported that determining the most appropriate form of treatment proves difficult. Another National focus group parent said, " . . . you need an integrated way to treat your child but there is nothing in place . . . there's no instruction manual or roadmap."

Inability to access necessary information about available treatment options was noted as a consistent, logistical barrier for parents. This was often described as a lack of appropriate information about available treatment options. Parents reported a lack of communication or support from their place of service, such as schools or their health providers. Other parents discussed long wait lists for appointments

to see psychiatrists or psychologists for evaluations. One parent expressed her frustrations with the lack of consistent options after receiving a diagnosis from a psychiatrist. This parent reported that she did not feel as if she had access to information about the correct direction to take post-diagnosis. She noted,

> . . . I had never – where is this supposed to be happening? So, that was my experience with it. I was very frustrated. And I'm still frustrated that, as a parent, when you hear this, what are you supposed to do? (National focus group)

Many parents described feeling as if they were educating themselves on available behavioral treatment options, as opposed to information and education being readily available from their place of service (POS). One parent went as far as to enroll in a psychology course to educate herself on treatment options for youth with ADHD and attended an ADHD conference but still reported frustration. She said,

> I jumped into trying to learn everything I could. Went to CHADD[2]; read all the materials. And I kept seeing these references to you need to do medication plus behavioral therapy . . . what CHADD seemed to be referring to was more along the lines of helping parents and teacher, but parents in particular to use behavior modification techniques. And what I found very frustrating was, it's like, yes, but, who, but where . . . (National focus group)

Parents described that the lack of coordination is often further heightened due to aforementioned financial costs, communication issues with providers, or lack of overall support. One participant noted,

> The financial implications of it all and then the stress of it having to feel that you're making the right decision for your child, while trying to wade through what felt to me like an overwhelming amount of information and points of view. (National focus group)

Parents also described a lack of communication between the systems of care, which may impact treatment efficacy. A parent from the National focus group stated, " . . . it's the types of things that fall into this behavior group [that] become really difficult because it involved multiple systems but none of those systems talk to each other." Another participant likened access to therapy as a "multi-problem approach," describing that her child required varied solutions to treat co-occurring behavioral and mental health issues. She noted,

> So we have to- parents have to piece it all together. I don't know whether to go to a psychologist, a psychiatrist, a pediatrician, a neurolo[gist]–I don't know who to go to to start. And so, you get all these little fragmented pieces and you just try to patch it all together for the remedy. (Southeast focus group)

Another parent in a Southeast focus group described care coordination among providers as the "missing piece" to help connect the best treatment options for her child, and said,

> I think that's part of the piece that's missing is . . . all of the different providers working together. I found this ADHD coach that worked with a psychologist, a psychiatrist—they all worked together. Every week they'd talk about her patients and how we can help them . . . there's not much of that around. And the ones that are around, they don't take insurance. (Southeast focus group)

One participant noted how treatment and care for children with ADHD are not triaged like other diagnoses. Comparing available treatment plans to more acute care, one parent stated,

> It's not like there's a diagnosis and a treatment plan, like cancer . . . you check after so many months, after so many years. There's not – no size fits all. (National focus group)

Parents of young children with ADHD diagnoses in the Southeast reported similarities in lack of direct referrals for treatment options, including upon diagnosis and while attempting to access different treatment options for their child. When asked if any health providers, pediatricians, or other medical professionals provided any direct referrals for services, parents in one Southeast focus group answered with a resounding "no."

Discussion

This study sought to highlight parent experiences and perceived barriers to behavioral health services for children diagnosed with ADHD. Responses from parents are consistent with prior research on barriers to ADHD treatment experienced by parents in the United States, including stigma, overall awareness and knowledge of ADHD, issues locating support and services, and logistics related to managing and navigating care (McKenna et al., 2023). In addition, our study also noted barriers with messaging and coordination from providers regarding behavioral health treatment (Mofokeng & van der Wath, 2017). Our study expands on the small number of previous qualitative studies on treatment barriers; our findings confirmed themes for barriers of behavioral service access that had emerged from studies of broader ADHD treatment access (McKenna et al., 2023). Despite being a treatment recommendation, major barriers exist to implementing behavioral treatment for young children diagnosed with ADHD as part of the evidence-based treatment protocol for children of all ages (Kappi & Martel, 2021). Parents described a need for providers – and places of service – to promote a more cohesive and unified treatment plan for young children diagnosed with ADHD. While major strides have been made with

increased representation and access for ADHD treatment options, stigma was a main barrier reported by parents. Misconceptions and stigma connected to an ADHD diagnosis often preempted parents' decision-making related to treatment, which has been identified as a theme in the literature (dosReis et al., 2010; McKenna et al., 2023; Paidipati et al., 2022). Parent responses in the current study illustrate the sociopsychological effects of misconceptions and stigma associated with receiving a health diagnosis (Goffman, 1989). Parents reported an initial awareness of their child's symptoms (i.e., hyperactive and emotional) without acknowledging it as deviant or impairing; they described how this further influences their decision-making by seeking available treatment. This process follows a similar pattern where parents of children with ADHD initially engage in normalization and hesitation, followed by concerns about fear and stigmatization (Spencer et al., 2021). Healthcare professionals as well as education professionals can provide information and education about when behavior is normative and when it is a cause for further evaluation to increase the likelihood that parents seek timely diagnosis and treatment. Further, professionals can address concerns regarding stigma and labeling to promote collaboration between parents and professionals on the treatment plan and to assuage anxieties and frustrations reported by parents (Wolraich et al., 2019).

Parent responses also highlight the amount of time required to participate in recommended behavioral therapies, consistent with prior research identifying time and other logistical challenges as prominent barriers to accessing treatment (McKenna et al., 2023). The added time requirement adds stress to their ability to provide care for their families. Parents described a time burden from navigating behavioral treatment options that result in missed time spent with family and career responsibilities. Participating in recommended behavioral therapy services for ADHD becomes an added stressor on parents in addition to being a possible solution to behavioral challenges. Additional treatment options, such as offering online parent training, may decrease barriers for some families (DuPaul et al., 2018). Considering most parents involved in the focus groups were women, more research is needed to identify whether these responses were further influenced by maternal stress (Blum, 2007) and whether there are unique barriers to treatment for mothers of young children with ADHD diagnoses.

Additionally, parents of children diagnosed with ADHD between 2 and 7 years of age in the Southeast alluded to more stigmatized experiences than parents in the National focus group. Parents in the Southeast particularly addressed concerns with stigma related to the child's behavior and to the ADHD diagnosis. Parents in the Southeast focus groups more often reported extensive issues navigating the requisite systems of care to determine the best treatment options, including lack of availability and access to appropriate treatment and referrals. These parents described feeling like pieces were "missing" regarding how to access treatment options. This aligns with national data pointing to higher prevalence of ADHD diagnoses in the states in the south of the U.S., but lower initiation of behavioral health treatment compared to the western states (Danielson et al., 2022; Visser et al., 2014). However, the fact that the Southeast focus groups were conducted in a community-based setting and the National focus groups were drawn from parents who attended a national conference on ADHD limits the generalizability of these differences.

Ultimately, parents described lack of information about their child's treatment as a barrier, regardless of their child's age. Parents reported having concerns about their child's behavior at the onset of a diagnosis but were unsure how to implement the appropriate treatment options. Parent training in behavior management is recommended by providers as the primary treatment option for preschool children, but many parents reported needing multiple treatment options to best fit their child's needs, consistent with prior research (Lu et al., 2022). The high burden and low support described by parents in the focus groups are particularly problematic given the chronicity of ADHD and the need for long-term treatment, specifically for children who experience symptoms at a young age (Loe & Feldman, 2007). The subcommitte on children and adolescents with attention-deficit/hyperactivity disorder within the American Academy of Pediatrics (Wolraich et al., 2019) identifies ADHD as a long-term condition for children that requires careful monitoring and ongoing support. In addition to facing issues with access to providers, parents also noted issues with a lack of coordination of care and described how the lack of consistency with treatment plans can cause ADHD treatment to become a "multi-problem approach." ADHD treatment approaches may benefit from having more robust and intentional treatment plans that include collaborations among providers and referrals to specialists for needed care, as well as collaboration with the schools, to develop treatment plans that are unique to their patient's lived experience (Leslie et al., 2007; Lu et al. 2022; Wolraich et al., 2019). Behavioral therapy is recommended for ADHD, yet clear and accurate messaging, education, support, and guidance regarding these treatments and how they may be integrated with medication where appropriate were largely reported as absent; the lack of consistent communication and coordination were described as frustrating for both parents and their children (Mofokeng & van der Wath, 2017). Pediatric care providers may benefit from additional training to address these barriers.

Limitations

Key limitations to this study are related to convenience sampling of focus group participants, the age of the data,

the inclusion of parents with older children with ADHD diagnoses, and the small number of participants recruited, resulting in limited, non-generalizable participant responses. Focus group participants were recruited from select groups of parents of children with ADHD (ADHD parental support group and ADHD-themed conference), which introduces sampling bias and significantly limits the generalizability of these findings. Parents recruited from both locations may have increased knowledge of ADHD treatment options or be more attuned to navigating care or seeking support. This limits generalizability and may have led to underestimation of barriers, as other parents who are not already connected to ADHD education and support may experience additional barriers. Future research that recruits both parents with presumed access and parents lacking access to a support group or ability to attend a conference may provide a more representative sample. The eligibility criteria for focus groups were also expanded beyond those in the focal age group (diagnosed between the ages of 2–7 years) to increase the pool of eligible participants while focusing on the treatment options offered to parents when receiving an *initial* ADHD diagnosis. This resulted in a sample of parents where over a third (43%, $n = 11$) of children were 11 years of age or older.

Factors that enable engagement in the select groups from which participants were recruited, and/or factors associated with agreeing to participate in this study, may also have been related to experiences accessing evidence-based treatment for their children in ways that prevent extrapolation of study findings to parents of children with ADHD more broadly. While many key themes emerged (both across parent responses in the two Southeast focus groups and between the National and Southeast focus groups), a larger sample size may be more conducive to generalizability and would support a better understanding of how certain factors influence diagnosis experiences. A more representative sample of parents of children with ADHD throughout the United States (outside of the Southeast) may lead to different conclusions. Additionally, not all participants in this study had children with the same length of time since diagnosis; parent recall with diagnosis or treatment experience may be a contributing limitation.

Race and ethnicity of children and parent participants were not captured at the time of the focus groups. While the research questions were not developed with the intention for across-group analyses based on race or ethnicity (or to identify differences with care), this is a limitation given that children of color are disproportionately impacted by misdiagnoses and face additional and markedly different barriers to receiving and maintaining care (Kamimura-Nishimura et al., 2023; Morgan et al., 2013, 2015; Zablotsky et al., 2020). Incorporating these demographic markers in future research would contribute to the literature on childhood ADHD diagnosis experiences across race, the potential misdiagnoses (and dual diagnoses) of ADHD among children of color (Ballentine, 2019), and potential additional barriers to receiving and maintaining ADHD treatment due to negative healthcare experiences. Increasing the sample size to allow and prioritize inclusion of children and parents of color would enable an understanding of their lived experiences and the barriers to receiving and managing ADHD treatment.

Finally, the age of the focus group data (2015) is a potential limitation. However, given that findings support and build on themes in the literature, our study findings are applicable in the current context.

Conclusion

The parent responses presented in this paper can provide a deeper understanding of perceived barriers to behavioral health services for young children diagnosed with ADHD. Our study contributes to current literature by highlighting barriers parents experience when attempting to initiate behavioral health services. The key themes identified in parent responses have implications for enhancing coordinated care of ADHD treatment options across places of service, providers, and families. Training of healthcare providers and educators, along with school-based intervention, may be viable options to improve both access and communication between services and parents, as well as decrease negative educational outcomes (Loe & Feldman, 2007; Wolraich et al., 2019). Addressing these barriers may further improve the alignment between current clinical practice and best practice for treatment of ADHD, particularly among young children. Further, parents from the two Southeast focus groups in particular reported barriers to access especially navigating appropriate care. The parent experiences and perceptions documented in this paper may also inform future efforts to improve access to evidence-based treatments for ADHD in the Southeast and nationwide.

Author Note

The findings and conclusions in this report are those of the authors and do not necessarily represent the official position of the Centers for Disease Control and Prevention.

Acknowledgments

The authors of this study would like to thank the parents who participated in the four focus groups. The authors would like to also thank the Centers for Disease Control (CDC) researchers Sue Visser; Lauren Whitney-Forbes and Akilah Rashida Heggs, who assisted with coding; and Haley Elia, a student intern. Director Hermoine Wellman-Relick was instrumental with organizing various meetings, events, recruitment, and logistical support with the 2016 Children and Adults with Attention Deficit Hyperactivity Disorder (CHADD) conference. The research team also thanks Joan Teach, PhD, the Educational Consultant and Director of the Community Resource Center in Atlanta, GA, who helped with recruitment and logistics for the Southeast-based focus groups.

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Ethical Considerations

This study was reviewed and approved by the Institutional Review Board at Georgia State University (H17324).

Consent to Participate

All participants provided written informed consent prior to participating.

Funding

The authors disclosed receipt of the following financial support for the research, authorship, and/or publication of this article: This work was supported by the FY2015 Georgia State University/ Centers for Disease Control & Prevention Seed Grant Program.

Declaration of Conflicting Interests

The authors declared no potential conflicts of interest with respect to the research, authorship, and/or publication of this article.

Data Availability Statement

The data underlying this article cannot be shared publicly to protect the privacy of individuals that participated in this study. The data can be shared on reasonable request to the corresponding author assuming the request is aligned with the study's informed consent language.

Notes

1. A community service board attempts to provide various forms of support to those seeking resources for mental health, substance use or recovery, and other forms of social assistance.
2. Refers to the non-profit organization Children and Adults with ADHD.

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